FOLFIRINOX followed by chemoradiotherapy · rectal cancer

This is a planned course before surgery assessment for locally advanced rectal cancer. Six FOLFIRINOX cycles are followed by capecitabine with pelvic radiotherapy. The team reviews response and the next part of your overall rectal-cancer plan.

Locally advanced rectal cancer · NCCP 00691 v4 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

My overall treatment

Phase 1: six FOLFIRINOX cycles, one every 14 days, each with a 46-hour fluorouracil pump. Phase 2: capecitabine twice daily on Days 1–5 of each seven-day week for five weeks, with radiotherapy. Your team supplies the separate radiation calendar.

14 day view

Treatment timing in this window

Day-ward medicines · induction — Day 1

Fluorouracil pump — Day 1–3 · 46 hours; disconnect as arranged

Phase 1 · six FOLFIRINOX cycles

FOLFIRINOX every 14 days for six cycles; each includes a 46-hour fluorouracil pump.

Phase 2 · five weeks with radiotherapy

Capecitabine twice daily on Days 1–5 of each seven-day week for five weeks, with radiotherapy. The radiation unit provides the appointment calendar; the team confirms any treatment-day changes.

Two phases with separate calendars

Phase 1 · cycles 1–6	Phase 2 · weeks 1–5	After the two phases
Every 14 days. Day 1 infusions then 46-hour pump.	Capecitabine twice daily Days 1–5 of each seven-day week with radiotherapy.	Team-directed restaging and surgery/ next-treatment assessment.

Your team confirms the active phase and dates. Symptoms may occur during or after treatment; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) / baseline bloods** Required by regimen	DPD before fluoropyrimidine; blood count, kidney and liver tests.
During	**Bloods / nerves** Required by regimen	Before each 14-day induction cycle and each weekly capecitabine cycle. Nerve review each induction cycle.
If indicated	**Heart** Conditional tests	electrocardiogram (ECG) if cardiac function is compromised.
After / review	**Recovery / scans / markers** Team-agreed plan	Follow the individual cancer plan and any unresolved effects; scan and marker intervals are team-agreed.

Between treatments

- No guaranteed safe days: report fever, sudden illness, diarrhoea or poor intake promptly.
- Symptoms can continue after pump removal and after treatment ends.

Confirm supportive medicines, tests, working urgent contacts and pump disconnection.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Mouth / swallowing

Fluorouracil + folinic acid / later capecitabine

Report ulcers or pain that affects eating or drinking.

3 Bowel / stoma

Irinotecan / fluorouracil / radiotherapy

Call early for diarrhoea, increased stoma output or poor intake; severe pain needs urgent assessment.

5 Blood / infection

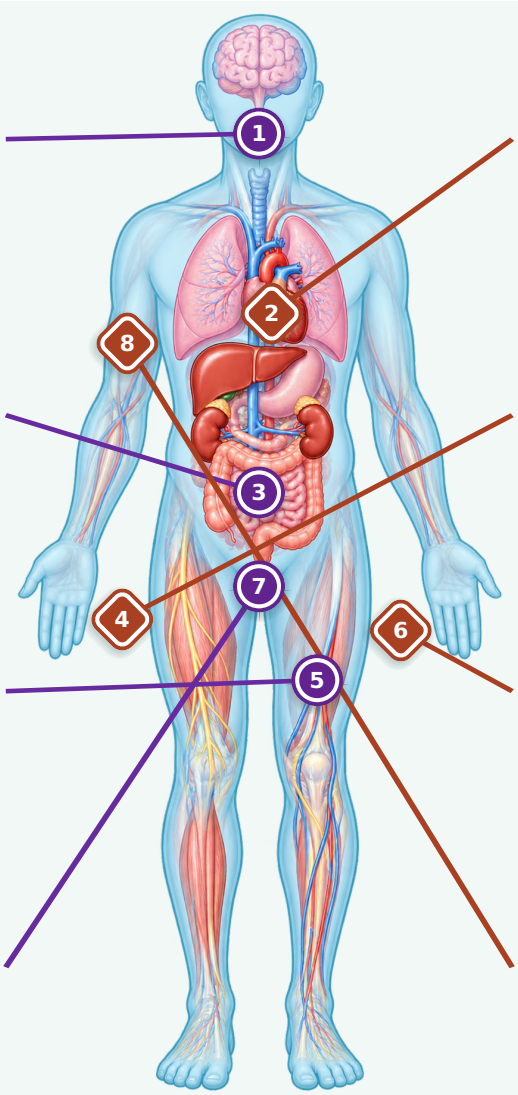
Chemotherapy

Fever, shivers or sudden illness: call now. Unusual bruising or marked weakness also needs advice.

7 Pelvis / bladder

Pelvic radiotherapy

Report painful urination, worsening pelvic symptoms or sore treatment-area skin.



2 Chest

Fluorouracil / capecitabine

New chest pain, collapse or major breathing difficulty needs emergency help.

4 Nerves / cold

Oxaliplatin

Avoid cold triggers; report persistent numbness or difficulty with buttons or walking.

6 Hands / feet

Capecitabine / fluorouracil

Painful redness or peeling affecting activity: hold capecitabine and call during the tablet phase.

8 Pump / line

Treatment delivery

Call for leakage, swelling, line pain or a loose connection. Do not reconnect it yourself.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or sudden illness needs immediate oncology advice. Chest pain, collapse or severe breathing difficulty needs emergency help. Call urgently for severe or persistent diarrhoea, severe abdominal pain, inability to drink, reduced urine or a line/pump problem.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

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Keep your treatment details to hand

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Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

FOLFIRINOX → rectal chemoradiotherapy

6 × 14-day FOLFIRINOX (46-hour pump), then 5 weekly capecitabine + radiotherapy cycles.

Phase: FOLFIRINOX induction · 6 cycles

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

FOLFIRINOX cycle record

Stamp the six induction cycles only.

C1	C2	C3
Date / initials	Date / initials	Date / initials
C4	C5	C6
Date / initials	Date / initials	Date / initials

My treatment information

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Chemoradiation record

Separate phase: team records each completed capecitabine/radiotherapy week. Follow the prescribed radiation calendar.

Week 1 _____ 2 _____ 3 _____ 4 _____ 5 _____ (date / initials)

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.